

THIRD SESSION – TENTH PARLIAMENT

REPORT OF THE PORTFOLIO COMMITTEE ON HEALTH AND CHILD CARE ON THE PETITION FROM CHRISTIAN LEGAL SOCIETY OF ZIMBABWE ON COMMUNITY CARE GIVERS.

PRESENTED TO THE NATIONAL ASSEMBLY

FEBRUARY, 2026

ORDER OF APPOINTMENT FOR PORTFOLIO COMMITTEE

ORDERED IN TERMS OF STANDING ORDER NO.17:

At the commencement of every session, there shall be as many Committees to be designated according to government policy as the Standing Rules and Orders Committee may deem fit. Each select committee shall be known by the portfolio determined for it by the Standing Rules and Orders Committee.

TERMS OF REFERENCE OF PORTFOLIO COMMITTEES

STANDING ORDER NO. 20

Subject to these Standing Orders, a Portfolio Committee shall-

1. examine expenditure administration and policy of government departments and other matters falling under their jurisdictions as Parliament may, by resolution determine.
2. consider and deal with all bills other than a Constitutional Bill, and statutory instruments or other matters which are referred to it by or under resolution of the House or by the Speaker;
3. consider or deal with an appropriation or money bill or any aspect of an appropriation or money bill referred to it by these Standing Orders or by or under a resolution of this House;
4. monitor, investigate, inquire into and make recommendations relating to any aspect of the legislative programme, budget, policy or any other matter it may consider relevant to the government department falling within the category of affairs assigned to it, and may for that purpose consult and liaise with such department; and
5. consider or deal with all international treaties, conventions and agreements relevant to it, which are from time to time negotiated, entered into or agreed upon.

SPEAKER'S ANNOUNCEMENT

On Wednesday, 18 October, 2024, the Speaker announced that the following Members shall serve on the Portfolio Committee on Health and Child care;

Hon. Dr. Khupe T.
Hon. Bajila D.
Hon. Bhila R.
Hon. Chakukura A.
Hon. Dumbarimwe T.
Hon. Hlatwayo C.
Hon. Kapoikulu S.
Hon. Karenyi L.
Hon. Nkala D.
Hon. Dr. Makumbe J.
Hon. Matsunga S.
Hon. Mahlangu S.
Hon. Molokele D.
Hon. Muchemwa W.
Hon. Muwodzeri T.
Hon. Murombedzi M.
Hon. Mutandi G.
Hon. Ndudzo P.
Hon. Nhari V.
Hon. Nyamupinga Karimatsenga B.
Hon. Ndlovu Nomathemba
Hon. Shongedza E.
Hon. Pinduka T.
Hon. Tobaiwa J.
Hon. Zhanda W.
Hon. Zhou P.
Hon. Ziyambi S.

Hon. Dr. Khupe T. to be the Chairperson

1.0 Introduction

In accordance with the provisions of Section 149 of the Constitution of Zimbabwe, the Christian Legal Society of Zimbabwe (CLSZ) requested Parliament of Zimbabwe, through a petition, to consider enactment of a law to support care givers as one of the possible interventions to address the nature of community care support systems in Zimbabwe. The Petitioners being grieved that Zimbabwe had gone through a cycle of different scourge such as HIV&AIDS, Cholera and Covid-19 pandemics and natural disasters such as Cyclone Idai among many others. In such pandemics and disasters, communities have relied on support from family members without public resource support. Thus, the Committee considered their grievances, and conducted an in-depth analysis of different case studies culminating into this report.

2.0 Christian Legal Society of Zimbabwe prayer

2.1 The Petitioner (s) prayer was that, Parliament;

a. To consider or inquire into the possibility of enacting a law for the provision of State support for community care givers.
b. Enact a law to provide for mechanisms of state interventions on community care work that are in sync with the right to health enshrined in the Constitution section 76, such mechanisms being such as to provide for community care givers including care giving in a systematic and effective way.

3.0 Committee Objectives

3.1 In its inquiry, the Committee was guided by the following objectives;

1. To respond to the Petitioner's prayer;
2. To appreciate the operations of Community Care Givers/Village Health Workers; and
3. To offer policy recommendations, in relation to the request by Petitioners.

4.0 Methodology

4.1 To gather clarity on the petition, on 25 March, 2024, the Committee received oral submissions from Mr. Gerald Matiba, the Executive Director, Christian Legal Society of Zimbabwe. In an effort to appreciate the Ministry's position, the Committee also received oral submissions from Dr.

A. Maunganidze, the Permanent Secretary for the Ministry of Health and Child Care. The Ministry further submitted written response.

5.0 Committee Finding

5.1 Legal Framework for Community Care Givers/Village Health Workers

(*Community Care Givers is synonyms with Village Health Workers)*

5.1.1 Community care nursing is already catered for in the current structure or the Ministry of Health and Child Care organogram, through the Nurses at health posts and clinics. The current community care structure evolves around the Village Health Worker as primary contact, being the eyes and ears of public health challenges in the communities. The Village Health Worker reports to the nurses at the nearest local clinic or health post to attend to the health and nursing issues in the communities.

5.1.2 In the past, the government has been criticized for not recognizing Village Health Workers, leaving their welfare in the hands of development partners. However, the government through the Ministry of Health and Child Care has made strides to recognize the role played by Village Health Workers. In the 2025 National Budget, Village Health Workers appears as a stand-alone budget line, and are receiving an allowance of USD15, with possibilities of increase in the future.

5.1.3 The allowance is disbursed quarterly, through respective municipal finances. Although, the allowance is minimal, the principle is reflecting government commitment to Village Health Workers. There is need to facilitate 100% salary coverage to all village health workers and allow Salary Service Bereu (SSB) to assume payment of these allowances. The Ministry intends to increase the allowance from USD15 to USD50 in the 2026 National Budget.

5.1.4 The current legal framework that is Health Professions Act and Health Service Act recognize the Village Health Workers as community care nursing among other health professionals.

5.1.5 The Petitioner's prayer regarding enacting a law is fully covered in the current legal space and the government through Ministry of Health and Child Care and Treasury are already implementing a budget to support and compensate for the services rendered by Village Health Workers.

5.2 Home Based Care

5.2.1 The Petitioners recognized the importance of Home-Based Care especially during pandemic such as HIV&AIDS and Covid-19, where the sick solely depend on family members to offer the necessary support services to their

relatives. Faced with a scenario where a family member falls critically ill or is bed ridden, individuals are left with no choice but to forgo their employment. Hence, home based care can be equated to unpaid care work. Upaid primarily denotes that one is doing an activity but they do not receive a wage for it and care means that the activity seeks to save people and their well-being. In this context, the right to health and work symbolizes that the activity is a cost. However, it further means that, as there is a cost, this activity does not warrant compensation or a contractual obligation.

5.2.2 Noble as it may appear, the opinions expressed by both the Committee and the Ministry of Health and Child Care, find the submission not practical in the current legal and economic landscape, therefore the matter was understood in the context of social safety nets and the Ministry of Public Service, Labour and Social Welfare to develop a strategy to manage Home Based Care initiatives.

6.0 Committee Observations

The Committee deliberated on the finding and observed the following;

1. The legal frameworks pertaining recognition of community care givers/village health workers are well catered within the current legal framework (Health Professions Act and Medical Services Amendment Act) where they are recognized as part and parcel of the medical profession.
2. The government through Treasury and Ministry of Health and Child Care have shown its commitment to compensate Village Health Workers. However, the allowances remain a drop in the ocean.
3. Compensation of individuals providing home-based care as reflected in the petition, would be resolved by improving social safety nets within the Ministry of Public Service, Labour and Social Welfare.

7.0 Committee Recommendations

The Committee made the following recommendations;

7.1 The Ministry of Health and Child Care in conjunction with Treasury should provide budgetary provisions for all the Village Health Workers in the 2026 National Budget and increase the allowance to USD100.00.

7.2 Treasury and Ministry of Health and Child Care should only disburse Village Health Workers' allowances under the SSB, to guarantee their allowances monthly.

7.3 Jointly, the Ministry of Public Services, Labour and Social Welfare and Ministry of Health and Child Care should develop a strategy to compensate or assist individuals and families under Home Based Care to easy their challenges by end of December 2026.

8.0 Conclusion

Village Health Workers plays a crucial role in primary healthcare delivery, acting as a vital link between communities and the formal health system. They are the first point of contact for many rural and peri-urban populations, providing essential health services and information. Although the government has shown commitment to compensate them, the allowances remain minimal and a cause for concern. Home based care, an important integral part of health delivery, ethics and values demand that family take care of one another in times of need. The costs associated are rather matters of social safety nets that the Ministry of Public Services, Labour and Social Services should consider.